# Ear Examination — OSCE Checklist

Deakin MD2 · BB4 · ENT

*Tick each step as you perform or verbalise it.* **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Inspect → Palpate → Otoscopy → Whisper test → Weber → Rinne**. *Examine the* **good ear first**.

## 1. Introduction

| | |
|---|---|
| ☐ | Wash hands; introduce self; confirm patient ID |
| ☐ | Explain the exam and gain consent; ask about pain |
| ☐ | Ask which ear is affected; position seated, ear at your eye level |
| ☐ | Equipment: otoscope with range of speculae, 512 Hz tuning fork |

## 2. Examination

| | Step | What to do / say | Abnormal |
|---|---|---|---|
| **Inspection (external ear)** | | | |
| ☐ | Pinna | Size, shape, deformity; trauma (haematoma) | Cauliflower ear |
| ☐ | Pre-auricular region | Cartilage tags, sinuses or pits | Syndromes with hearing loss |
| ☐ | Skin | Suspicious lesions (SCC, BCC), skin conditions of pinna and canal | Neoplasia, eczema |
| ☐ | Scars | Pre- or post-auricular | Previous surgery |
| ☐ | Hearing aids | Note and remove before otoscopy | |
| ☐ | Discharge | Mucopurulent, serous, blood-stained | Otitis externa / media, perforation |
| **Palpation** | | | |
| ☐ | Tragus and pinna | Gently move — tenderness? | Tender = otitis externa |
| ☐ | Mastoid | Palpate behind the ear | Tender = mastoiditis (with otitis media) |
| **Otoscopy** | | | |
| ☐ | Speculum | Largest that fits comfortably | |
| ☐ | Grip | Hold otoscope like a pen; hand braced against patient's head | Prevents injury if head moves |
| ☐ | Straighten canal | Adult: pull pinna up and back; child: pull back only | |
| ☐ | Canal | Inflamed? Wax? Discharge? Foreign body? | Otitis externa |
| ☐ | Tympanic membrane | Colour, light reflex, short process and handle of malleus, umbo, pars flaccida; move to see whole TM | Bulging, retraction, perforation, grommet |
| **Hearing** | | | |
| ☐ | Whisper test | Mask other ear (rub tragus / fingers); whisper numbers; increase volume if needed; each side | Formal audiology if abnormal |
| ☐ | Weber (512 Hz) | Strike fork; base on vertex or midline forehead: "Louder on one side or the middle?" | Lateralises (see table) |
| ☐ | Rinne (512 Hz) | Base on mastoid until no longer heard, then prongs beside the ear: "Can you hear it now?" | Positive (AC > BC) = normal |

## 3. Closing

| | |
|---|---|
| ☐ | Thank patient; clean otoscope speculum; wash hands |
| ☐ | Present: "Pinnae normal, no tenderness; canals clear; both TMs intact and pearly grey with a normal light reflex; whisper test normal; Weber central, Rinne positive bilaterally." |
| ☐ | Completion: "I would complete the ENT exam (nose, throat, neck nodes) and arrange formal audiometry if indicated." |

## 4. Quick reference

| Weber / Rinne (Deakin table) | Weber central | Weber lateralises LEFT | Weber lateralises RIGHT |
|---|---|---|---|
| Rinne AC > BC both ears | Normal | Sensorineural loss RIGHT | Sensorineural loss LEFT |
| Rinne BC > AC left | — | Conductive loss LEFT | Combined loss |
| Rinne BC > AC right | — | Combined loss | Conductive loss RIGHT |

| TM appearance † | Think of |
|---|---|
| Pearly grey, light reflex | Normal |
| Red, bulging, lost landmarks | Acute otitis media |
| Dull, retracted ± fluid level / bubbles | Otitis media with effusion |
| Hole in TM | Perforation |
| Small tube in TM | Grommet (not a foreign body) |
| Swollen canal, debris, tender tragus | Otitis externa |

- **Conductive** loss: Weber goes TO the bad ear, Rinne negative (BC > AC).
- **Sensorineural** loss: Weber goes AWAY from the bad ear, Rinne positive (both reduced).
- **False-negative Rinne**: a dead ear hears BC louder via the other cochlea — Weber clarifies.
- Rinne negative needs a conductive loss of ~20 dB or more.